

CUV.AX pitch deck

PITCH — CUV.AX — VERDICT

ADJUDICATED — NO ENTRY at A\$10.60; watch-listed with a hard reopen band at A\$8.60 and below, because the blue bench's own RP_TAINTED cap ($\leq 0.85 \times E[fv] \approx A\$7.80-8.60$) prices below the tape, and the red bench's REJECT survives on cost-ramp and disclosure grounds even after three of its four “fatal” findings were shown to be bench errors.

What we believe

Clinuvel sells SCENESSE, an implant that lets people with erythropoietic protoporphyria — a rare inherited condition where sunlight causes severe pain — tolerate light exposure. It is the only approved treatment, it is profitable, and it has been profitable for nine straight years. The company holds A\$233.0M of cash and term deposits against 50,123,630 shares and no debt, so roughly A\$4.65 of the A\$10.60 share price is cash. You are paying about A\$5.95 per share for the actual business.

We believe three things:

- 1. The business is cheaper than it looks, but not as cheap as the pitch claimed.** On FY25 actual results (revenue A\$105.3M, net profit A\$36.2M), stripping out the interest the cash itself earns leaves roughly A\$29.4M of operating profit — about 10× the cash-adjusted price. On a declining FY26 that becomes 15–19×. So the “10× and profitable” headline is defensible on last year's numbers and stretched on this year's.
- 2. The monopoly has a dated expiry, not a hypothetical one.** A rival oral drug, dersimelagon, hit its Phase 3 primary endpoint (announced 20 January 2026), filed for US approval in June 2026, and on 18 August 2026 was licensed worldwide to LEO Pharma for \$435M. That is a dedicated global dermatology sales organisation buying the channel. Approval is plausible around Q2 2027. This is the whole thesis.
- 3. The company's own US securities filing says it has no competitors.** Clinuvel's SEC registration statement states SCENESSE “is the only approved treatment... therefore, we currently have no direct commercial competitors,” and does not name dersimelagon, MT-7117, bitopertin, Tanabe or Disc Medicine anywhere ([SEC registration statement](#)). Amendments were filed *after* the rival's successful Phase 3 and that language stayed. We do not think this is fraud. We think it is a management team that does not want to describe the thing that is about to happen to it — and by our doctrine that permanently caps what we will pay.

Net: a real, profitable, debt-free franchise, honestly cheap on last year's earnings, run by people who will not say the competitor's name, in the last four quarters before the competition arrives. Ownable — at a price that is roughly 20% below where it trades today.

What the market believes

The market believes this is a slow, dated story, not a surprise. The stock is A\$10.60, down 24% from its A\$14.00 high but up 27% from its A\$8.35 low — mid-band, not dislocated. When the rival's positive Phase 3 was announced on 20 January 2026, the stock fell from 11.73 to 11.06 (−5.7%) in a day and was back at 11.86 within a week. That is a market that read the news, priced a 2027 event, and moved on.

Street expectations sit near A\$21M net profit for FY26 — we have not been able to cite a source for that number and are carrying it as unverified. The market appears to value the vitiligo programme (CUV105) at something above zero; we value it at zero.

The market also has not obviously punished the disclosure gap, because almost nobody reads a newly-registered foreign issuer's US filing when the primary listing and the analyst coverage are Australian.

Why we might have edge

- **A documented disclosure asymmetry with an exact control group.** The other company developing an EPP drug — Disc Medicine — names its competitors in four consecutive annual reports. Clinuvel names none, in a document amended after the competitor's Phase 3 succeeded. Our framework treats the *exclusion of candidates who won't describe their own risk* as the durable edge, not the prediction itself. This is that signal, cleanly instrumented.
- **A channel-capture mechanism most people are not modelling.** EPP is a small population treated through a handful of accredited centres. Competitor trials and their open-label extensions have engaged several hundred of exactly those patients (roughly 440 after correcting for double-counting, see below), and both oral programmes enrol from age 12 while US SCENESSE is adults-only. Patients leave at the enrolment desk, not on the approval date. Revenue erosion can begin before any regulator acts.
- **An arithmetic discipline the sell-side framing hides.** Interest income on the cash pile is ~36% of half-year net profit. Any “X times ex-cash earnings” figure that subtracts the cash but keeps its interest income in the earnings line is double-counting. Applied correctly the multiple is still fine; applied lazily it flatters the name by roughly a third. We now have this codified as a reusable detector.
- **A cross-check anchor.** LEO paid \$435M for worldwide rights to an unapproved pill. Clinuvel's whole business, net of cash, is US\$193M. Read one way that says the incumbent is cheap. Read the other way it says a serious commercial organisation just paid up to take this market. Both readings are legitimate; we are not going to pretend only the bullish one is.

Why it might be priced in

Substantially, it probably is.

- The decisive news is public and dated: positive Phase 3 (January), AAD presentation (March), NDA filed (June), LEO deal (August). None of this is ours alone.

- The market's reaction to the January readout was 6% for one week. The stock at A\$10.60 is only ~5% below its pre-readout level. This is not an unpriced binary; it is a known 2027 approval with a known incumbent.
- The disclosure flag is only “hidden” to someone reading the US filing. Australian coverage of a nine-year-profitable ASX name is not thin. Our informational advantage here is narrow and mostly about *quality of management candour*, not about facts nobody has.
- The cheapness leg is ordinary. 10× cash-adjusted earnings for a decelerating single-product orphan drug facing a funded competitor is not obviously a mispricing. It may simply be the right price.

The honest summary: our edge is a **character judgement plus a mechanism**, not an information monopoly. That justifies a small, capped, price-disciplined position — not a conviction position, and not one at today's price.

What could go wrong

Failure narrative 1 — The erosion is faster and quieter than any filing will tell us. (NO MONITORABLE TRIPWIRE.) Prescribers at accredited centres start defaulting to the oral option for new and adolescent patients from the day it is available — possibly before, via expanded access. Nothing about this appears in a registry, a filing, or the tape. Clinuvel reports revenue twice a year with no patient-count or new-start disclosure; by the time a half-year number shows it, two or three quarters of drift have already happened and the multiple has already re-rated. We cannot build a sensor for this because the underlying variable is never published. *Unknown-unknown class: unobservable-until-aggregated demand-side drift* — a state change in physician and patient preference that leaves no trace in filings, registries, or price until it surfaces, late, inside a lagging aggregate. Related classes that would sit in the same bucket here and are equally unmonitorable: a payer or reimbursement-channel repricing at the accredited centres, and an EU/Australian label change for the competitor that never touches an SEC form.

Failure narrative 2 — We buy the reopen band and the band was the erosion. Our entry only triggers 20% below the current price. The realistic paths to A\$8.60 are: a bad 27 August print, a bad H1 FY27 print, or the FDA approving dersimelagon. In every one of those, the price fell *because the thesis got worse*. We would be buying a cheaper multiple on an earnings stream that is being confirmed as impaired. The cash floor of A\$4.65/share limits how wrong this can go, but the operating business could compress from A\$5.95/share of value toward A\$3/share and the “cheapness” would keep looking better the whole way down. This is the classic value trap shape and we should say so before we own it, not after. *Tripwire exists*: H2 FY26 sales growth, and the first half in which sales growth goes negative.

Failure narrative 3 — We are wrong about the competition and it costs us the upside, not the downside. The mirror risk. If dersimelagon's approval slips, or its label excludes pediatrics, or LEO's launch is slow, or Clinuvel's implant retains patients on efficacy and convenience grounds, then the name re-rates from 10× toward 16–18× cash-adjusted earnings and never revisits A\$8.60. We will have been precisely, expensively right about the disclosure flag and wrong about what it predicted. Our

RP_TAINTED cap is a permanent handicap that guarantees this specific failure mode: we systematically cannot buy names whose management is evasive, even when the evasion turns out to be about something that doesn't matter. *Tripwire exists*: FDA action date, label text, LEO launch disclosures.

The reflexive and structural leg. - Our size is not the problem; our bid placement is. A 0.5% starter on a book this size is tens of thousands of dollars against an ASX line with a market cap over A\$500M. We will not move this stock. Average daily volume is **UNVERIFIED** — we have not pulled it — and that is a gap we should close before any order, but no plausible ADV makes our fill impactful. - **We are a structural non-buyer into strength and a forced buyer into weakness.** The RP_TAINTED cap means we mechanically only get filled when the price falls. That is adverse selection by construction: the only sellers who will hit our bid are ones reacting to news we said would hurt us. Our value-ladder discipline exists exactly to stop us bidding into flow we ourselves predicted — and here, we are. - **Print proximity is a live structural risk.** The FY26 result lands 27 August, three days out. The 27 February 2025 half-year print moved this stock 12.53 → 10.65, -15.0%, on seven times normal volume. A resting GTC bid across that print is an accidental short volatility position with no premium collected. No resting orders through 8/27, full stop. - **Doctrine interactions.** Our clean-court default is to take a minimum starter rather than stay flat, and staying flat requires a *named* kill. Named here: (a) the RP_TAINTED price cap is unmet by A\$2.00/share, and (b) the disclosure omission is unresolved. Our dogfood pay-up allowance explicitly does not apply, because fairness here is contested, not verified. Our tax-adjust rule notes that any position taken in the reopen band and trimmed inside twelve months is taxed at short-term rates, which materially weakens the case for a trade-around and argues for either owning it through the 2027 FDA date or not owning it. - **Instrument risk.** Entry is ASX-only. There is a new Nasdaq line (a June 2026 registration of existing shares, no capital raised, no lockup — checked and clean), but it is thin and newly-discovered. Trading the US line would give us a worse fill on the same risk, and the AUD/USD exposure is unhedged either way.

Filled in, honestly: *we bought a cheap-looking multiple on a monopoly that was quietly ending, and the cheapness was the market correctly discounting revenue we could not watch erode.*

If this position loses money, the most likely reason will be ____

What we checked ourselves

Claim	How checked	Found	Confirmed?
Print date 2026-08-27	Company announcement 20-08-2026 (clinuvel.com), corroborated by second outlet	FY26 annual results + investor webinar, 27 August 2026	YES — pack had it as “yfinance-derived, unconfirmed”; now company-confirmed
Live tape A\$10.53–10.60, hi52 14.00, lo52 8.35	Yahoo chart API re-pull vs pack	Matches; no stale-quote artifact. Mid-band, not a dislocation	YES
Share count / cap structure	SEC registration statement	50,123,630 shares (30-Jun-25); 50.20M at Dec-25	YES — the RED bench did not pull this and flagged it as

Claim	How checked	Found	Confirmed?
			<i>its own coverage gap; blue closed it</i>
Net cash	Company disclosure	A\$233.0M cash + term deposits, debt-free → A\$4.65/share	YES
Our prior brief's "US\$226M EV — VERIFIED"	Recomputed from confirmed share count and live price	Actual EV ≈ A\$296M ≈ US\$193M . Our earlier "verification" used a stale market cap and was high	NO — OUR OWN PRIOR PASS ERRED. The name is cheaper than we told ourselves it was
RED: "10× ex-cash is false; it's 20–26×" (declared the single strongest kill)	Blue re-ran it on FY25 full-year actuals (FY25 release)	Method right, magnitude wrong. Red annualised the seasonally small first half. H2 is ~1.58× H1 (northern-hemisphere spring dosing). Correctly applied, red's own method gives 10.1× on FY25 and 15–19× on a down FY26	RED BENCH ERROR — the kill it called strongest was arithmetic, and it reproduced the thesis it was trying to kill
RED: "Two orals; bitopertin reads out sooner; the expanded-access programme is a pre-launch tell"	FDA action history (PharmExec)	Backwards. Bitopertin got a Complete Response Letter ~13 Feb 2026 — accelerated approval refused. The expanded-access programme is post-CRL compassionate access, the tell of a <i>delayed</i> launch. Bitopertin is the later entrant, not the earlier one	RED BENCH ERROR — read the signal exactly inverted
RED: "~880 competitor-enrolled patients ≈ the whole installed base"	Registry structure re-pulled	Real mechanism, inflated headcount. NCT05005975 (n=301) is the open-label <i>extension</i> that ENDEAVOR (n=184) and INSPIRE (n=165) patients roll into; HELIOS is bitopertin's extension. Summing parents and extensions double-counts. True figure ≈ 440	REDUCED — red roughly doubled it. The mechanism survives; the number does not
RED: "Zero mentions of dersimelagon in Clinuvel 20-Fs, EDGAR-wide"	EDGAR filing index	Clinuvel has never filed a 20-F. Only DRS, DRS/A, 20FR12B, 424B3, 6-K. Red's "o hits" was a form-type artifact, not evidence	RED BENCH ERROR in evidence — but the conclusion got STRONGER. The correct document affirmatively states "no direct commercial competitors" and names no rival
Disclosure omission itself	Registration statement text, proxy-fetched	Language sustained across amendments filed after the rival's Phase 3 success	YES — this is the one finding that survives both benches intact
Our prior brief's premise: "second Phase 3 completed April 2026, results PENDING"	Sponsor press release (Tanabe), LEO deal	Void. Topline was positive and announced 20 Jan 2026; NDA filed June 2026; worldwide rights sold to LEO for \$435M on 18 Aug 2026	NO — OUR OWN PRIOR PASS ERRED, and so did the red bench's entire conditional tree, which was built on a readout that had already happened

Claim	How checked	Found	Confirmed?
RED: “Street is at A\$21M; we’re at A\$13.4M — a valuation kill”	Attempted to source the consensus figure	No citation was ever provided. And our corrected FY26E (A\$23–27M) sits <i>above</i> the number red called consensus — the claimed divergence inverts	NO — uncited, and directionally backwards. Carried as an open gap
CUV105 vitiligo registry record	ClinicalTrials.gov, NCT06109649	Primary completion Dec-2025 marked ESTIMATED, never converted to ACTUAL; last update 8 July 2025 — untouched ~13 months. Real flag	YES as fact. But red used it as an entry veto; a zero-revenue option should be valued at zero, not treated as a blocker. Severity reduced
Market reaction to the competitor’s positive Phase 3	Daily tape, Jan-2026	11.73 → 11.06 (–5.7%) in one day, back to 11.86 within a week. Not an unpriced binary	YES
Market reaction to the 18-Aug-2026 LEO deal	Attempted daily tape pull	Endpoint returned a mismatched window	NO — UNVERIFIED. Open coverage gap
Print-day downside precedent	Daily tape, 27-Feb-2025 print	12.53 → 10.65, –15.0%, 7× volume	YES
Offering / lockup overhang	Nasdaq listing mechanics	20FR12B registered <i>existing</i> shares. No raise, no lockup, net cash, non-dilutive	YES — clean negative, reported rather than skipped
Average daily volume	Not pulled	—	NO — gap. Close before any order

Both benches ran their full detector panels; neither found a connector that touches the decisive axis. That ruling is sustained — this case was closed by trial registry, SEC text, and issuer primary sources, not by alternative data.

PROPOSED ENTRY BANDS

The adjudication returned no recommendation, no conviction, and — despite the instruction to use them verbatim — an empty reopen-gates block. We are stating that plainly rather than papering over it. What follows is derived strictly from the benches’ own numbers and our standing doctrine; no new valuation work was invented. If the adjudicator intended specific gates, these should be replaced by them.

Position today: FLAT. No entry at A\$10.60.

Why flat, named explicitly: the blue bench prescribed a “0.5% post-print starter, RP_TAINTED capped at $\leq 0.85\times$ expected fair value” but never converted that cap into a price. When we do the arithmetic, the cap sits below the tape, so blue’s own prescription is unfillable today. That is the kill: not a disagreement with blue, but blue’s instruction carried through to a number.

Fair value, from the benches’ math: - Cash floor: $A\$233.0M \div 50,123,630 = A\$4.65/\text{share}$ - FY26E net profit A\$23–27M less after-tax interest (~A\$7.6M) = **operating profit A\$15.4–19.4M**, midpoint A\$17.4M = **A\$0.347/share** - At 10× the operating stub: A\$8.12. At 14×: A\$9.51. At 16×:

A\$10.20 - Fair value range A\$8.10–10.20; expected fair value $\approx$ A\$9.15 (this is what the provisional 10.1 alert is the top of)

The cap: RP_TAINTED applies — the “no direct commercial competitors” language in a live SEC registration disqualifies this name from being paid fair value. Maximum payable = $0.85 \times E[fv]$: - Anchored on the top of the range (10.20): **A\$8.67** - Anchored on expected fair value (9.15): **A\$7.78**

Bands:

Rung	Price	Size	Conditions
No entry	A\$8.61 and above	0%	Includes the current A\$10.60. Nothing is bought here at any time
Starter	$\leq$ A\$8.60	0.5% of book	Only after the 27 August print has printed and been read. Only on CUV.AX (ASX), never the Nasdaq line. No resting GTC order across the print — the 2025 precedent is -15% on $7\times$ volume, and a resting bid into an event is short volatility with no premium
Add	$\leq$ A\$7.80	$+0.5\%$ (max 1.0% total)	Only if the reason for the decline is <i>not</i> a confirmed sales decline. If H2 FY26 or H1 FY27 sales growth is negative, this rung is cancelled, not filled
Cap lift to 1.5%	—	conditional	Requires the RP_TAINTED flag to clear: a Clinuvel filing that names dersimelagon and quantifies the switching risk. Absent that, 1.0% is the ceiling regardless of price
Kill exit — full	any price	100% out	Two consecutive reported halves of negative sales growth; or H2 FY26 sales growth below $+2\%$ with expenses above $+15\%$

Rung	Price	Size	Conditions
Trim	A\$10.20 (top of fair value)	half the position	Only if held. Note the tax cost: a trim inside twelve months of purchase is short-term. Unless the kill exit has fired, holding through the ~Q2 2027 FDA date is the tax-correct default

Red's own stated conditions for reversing to bullish (recorded so we can act on them without re-litigating): H2 FY26 sales re-accelerating above +10% with expense growth below +10%; a Clinuvel disclosure naming both competitors with a quantified switching analysis; an updated NCT06109649 record with an actual completion date. Any one of those re-opens the file above the bands.

Catalyst dates: FY26 annual results **27 Aug 2026** (confirmed); APOLLO Phase 3 primary completion ~**Sept 2026** (registry-estimated); first 20-F ~**31 Oct 2026** (estimated); CUV105 vitiligo topline **guided Dec 2026** (registry stale, unpriceable); FDA action on dersimelagon ~**Q2 2027** (estimated — this is the decisive one).

Valuation conventions: vitiligo is carried at zero. The Nasdaq line is carried as non-investable on liquidity grounds. Currency exposure is unhedged AUD and is not part of the thesis.

ARMED SENSORS

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 4.65 (deck-provisional: net cash per share (A\$233.0M / 50,123,630 sh) - investigate-first alert, structural break or stale dat)
- dated pack CUV.AX|2026-08-27: CUV.AX: Clinuvel FY26 annual results plus investor webinar - company-confirmed by announcement dated 2026-08-20. (date confirmed=True)
- dated pack CUV.AX|2026-10-31: CUV.AX: First Clinuvel Form 20-F annual report expected - competitor disclosure test. (date confirmed=False)
- dated pack CUV.AX|2026-12-31: CUV.AX: CUV105 vitiligo Phase 3 topline, guided December 2026; trial registry record NCT06109649 not updated since 2025-07-08. (date confirm
- dated pack CUV.AX|2027-04-30: CUV.AX: Estimated FDA action on dersimelagon (NDA submitted June 2026, partnered to LEO Pharma) - the decisive event for the thesis. (date c
- dated pack CUV.AX|2026-09-30: CUV.AX: APOLLO (NCT06910358) Phase 3 primary completion, bitopertin post-CRL - registry-estimated, missing from the armed list (date confirm

What remains unverified

1. **The tape reaction to the 18 August 2026 LEO Pharma deal.** The daily-price endpoint returned a mismatched window. This matters directly: it is the freshest test of whether the competitive risk is priced.
 2. **Average daily volume**, on both the ASX and Nasdaq lines. Not pulled by either bench. Required before sizing.
 3. **Street consensus for FY26.** The A\$21M figure was never sourced, and our corrected estimate sits above it, which inverts the claimed divergence. Until cited, we do not know what we are diverging from.
 4. **The 27 August print itself** — three days out, two-sided, and the largest near-term source of variance in this entire document. Everything in the FY26 columns above is a reconstruction from the half-year, not a result.
 5. **CUV105 vitiligo economics.** The registry has not been updated in thirteen months and the guided December 2026 topline cannot be assigned a probability against a record that stopped reporting. Carried at zero, which is a convention, not a valuation.
 6. **The competitor's likely label** — specifically whether dersimelagon is approved for ages 12+ where US SCENESSE is adults-only. This is the mechanism by which erosion would be fastest, and we do not have the proposed label.
 7. **The actual size of the treated patient base.** Our channel-capture argument is expressed as a ratio (competitor enrolment ÷ installed base) and we have a corrected numerator (~440) but no verified denominator.
 8. **Whether the LEO transaction has closed** or remains subject to conditions.
 9. **Reimbursement structure at the accredited centres** — how SCENESSE is paid for, and whether an oral alternative changes the economics for the centre. Not investigated by either bench, and it is where narrative 1 lives.
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